

Pursuant to the authority vested in the Commissioner of Health by section 4403 of the Public Health Law, Part 98 of Title 10 (Health) of the Official Compilation of Codes, Rules and Regulations of the State of New York is amended to be effective July 1, 2025, to read as follows:

Subdivision (p) of section 98-1.13 is added to read as follows:

A MCO shall meet standards for network adequacy and access standards for mental health and substance use disorder treatment services set forth in Subpart 98-5.

Subpart 98-5 is added to read as follows:

Subpart 98-5 Network Adequacy and Access Standards for Mental Health and Substance Use Disorder Treatment Services

Section 98-5.1 Purpose.

Section 98-5.2 Applicability.

Section 98-5.3 Definitions.

Section 98-5.4 Network provider type standards.

Section 98-5.5 Appointment wait time standards.

Section 98-5.6 Access to providers for enrollees.

Section 98-5.7 Provider directory requirements.

Section 98-5.8 Additional MCO responsibilities regarding network adequacy and access.

Section 98-5.9 MCO reporting on network adequacy and access.

Section 98-6.0 Effective date.

## Section 98-5.1 Purpose

Part II of chapter 57 of the Laws of 2023 amended the Public Health Law to improve access to behavioral health services in this State. Public Health Law 4403(5)(b), as added by Subpart F of Part II, requires the commissioner, in consultation with the Superintendent of Financial Services, the commissioner of the Office of Mental Health, and the commissioner of the Office of Addiction Services and Supports, to propose regulations setting forth standards for network adequacy for mental health and substance use disorder treatment services, including sub-acute care in a residential facility, assertive community treatment services, critical time intervention services, and mobile crisis intervention services. Subpart A of Part II establishes the effective date for coverage of sub-acute care in a residential facility, assertive community treatment services, critical time intervention services, and mobile crisis intervention services. This Subpart implements the requirements of Public Health Law 4403(5)(b), as amended by Subpart F of part II of chapter 57 of the Laws of 2023, and the requirements of Subpart A Part II by establishing network adequacy and access standards and other protections to improve access to behavioral health services.

## Section 98-5.2 Applicability.

- (a) This Subpart shall apply to all MCOs offering coverage that are subject to the mental health and substance use disorder requirements under Insurance Law § 4303 and Public Health Law § 4406.

### Section 98-5.3 Definitions.

As used in this Part:

(a) *Appointment wait time* means the time from the initial request for health care services to the earliest date offered for the appointment for services.

(b) *Behavioral health services* mean mental health services and substance use disorder treatment services.

(c) *Health care professional* means an appropriately licensed, registered, or certified health care professional pursuant to title 8 of the Education Law or a health care professional comparably licensed, registered, or certified by another state.

(d) *Health care provider or provider* means a health care professional, or a facility licensed, certified, or designated pursuant to Public Health Law articles 28, or Mental Hygiene Law articles 19, 31, 32, or 36, or a facility comparably licensed or certified by another state.

(e) *Network* means the health care providers with which a MCO has contracted to provide health care services to enrollees.

(f) *Non-participating* means not having a contract with a MCO to provide health care services to an enrollee.

(g) *Participating* means having a contract with a MCO to provide health care services to an enrollee.

(h) *Telehealth* has the meaning set forth in section 2999-cc of the Public Health Law and includes audio-only visits.

#### Section 98-5.4 Network provider type standards.

(a) Pursuant to Public Health Law section 4403(5)(b), a MCO shall ensure that its provider network is adequate to meet the behavioral health needs of enrollees and provide an appropriate choice of providers sufficient to render the behavioral health services covered under its health insurance contracts.

(b) An adequate network of health care providers of behavioral health services shall include residential facilities that provide sub-acute care; assertive community treatment providers; critical time intervention services providers; and mobile crisis intervention services providers, after the commissioner, in consultation with the Superintendent of Financial Services, the commissioner of the Office of Mental Health, and the commissioner of the Office of Addiction Services and Supports, has determined, for each provider type listed in this subdivision, that there is a sufficient number of certified, licensed, or designated health care providers available in this State to meet the network adequacy standards established by Public Health Law 4403(5)(b). Once the commissioner makes this determination, the network adequacy standards shall apply to the provider types listed in this subdivision for contracts issued, renewed, modified, or amended 90 days after the commissioner makes this determination.

#### Section 98-5.5 Appointment wait time standards.

(a) A MCO shall ensure that its network has adequate capacity and availability of health care providers of behavioral health services to offer enrollees appointments with providers that can treat enrollees' behavioral health conditions within:

- (1) 10 business days for an initial appointment with an outpatient facility or clinic;

(2) 10 business days for an initial appointment with a health care professional who is not employed by or contracted with an outpatient facility or clinic; and

(3) seven calendar days for an appointment following a discharge from a hospital or an emergency room visit.

(b) A MCO may meet the appointment wait times set forth in subdivision (a) of this section through the use of telehealth unless the enrollee specifically requests an in-person appointment to treat the enrollee's behavioral health condition.

#### Section 98-5.6 Access to providers for enrollees.

(a) If an enrollee is unable to schedule an appointment with a participating provider of behavioral health services within the appointment wait times set forth in section 98-5.5 of this Part because there is not a participating provider of behavioral health services available within the appointment wait times who can treat the enrollee's behavioral health condition, the enrollee, or the enrollee's designee, may submit an access complaint by telephone, and in writing to the MCO to resolve the access issue.

(b) The MCO shall have three business days from receipt of the access complaint to locate a participating provider of behavioral health services that can treat the enrollee's behavioral health condition and is able to meet the appointment wait times set forth in section 98-5.5 of this Part and to give the enrollee or the enrollee's designee the name of and contact information for the provider or providers by telephone, if the request was made by telephone, and in writing. If the enrollee specifically requests an in-person appointment, the provider shall be located within a reasonable distance from the enrollee;

however, the distance may be greater for enrollees who reside in rural areas than for enrollees who do not reside in rural areas.

(c) If the MCO is unable to locate a participating provider of behavioral health services that can treat the enrollee's behavioral health condition, is able to meet appointment wait times set forth in section 98-5.5 of this Part, and is located within a reasonable distance from the enrollee if the enrollee specifically requests an in-person appointment, the MCO shall;

(1) notify the enrollee by telephone, if the request was made by telephone, and in writing, at the expiration of the time period in subdivision (b) of this section that the enrollee may obtain a referral to a non-participating provider at the in-network cost-sharing and include contact information for the New York State Behavioral Health Ombudsman Program; and

(2) approve a referral to a non-participating provider, regardless of whether the enrollee's coverage includes out-of-network benefits, if the non-participating provider:

(i) can treat the enrollee's behavioral health condition;

(ii) is able to meet the appointment wait times set forth in section 98-5.5 of this Part, as measured from the enrollee's receipt of the notification in paragraph (1) of this subdivision;

(iii) is located within a reasonable distance from the enrollee if the enrollee specifically requests an in-person appointment; and

(iv) charges rates that are not excessive or unreasonable.

(d) The approved referral shall remain in effect until the earlier of the following:

(1) the behavioral health services are no longer medically necessary; or

(2) the MCO locates a participating provider of behavioral health services that can treat the enrollee's behavioral health condition, is able to meet the appointment wait times set forth in section 98-5.5 of this Part and is located within a reasonable distance from the enrollee if the enrollee specifically requests an in-person appointment, and the enrollee's treatment can be transitioned to the participating provider, unless the MCO determines, in consultation with the enrollee's treating provider, as appropriate, that such a transition would be harmful to the enrollee. If the enrollee or the enrollee's designee disagrees with the MCO's transition of care determination, the enrollee or the enrollee's designee may request an expedited determination or appeal pursuant to Public Health Law section 4408-a or 4904, as applicable.

(e) The MCO shall not impose cost-sharing on the enrollee, including a copayment, coinsurance, or deductible, for the service rendered by a non-participating provider pursuant to an approved referral, that is greater than the cost-sharing that the enrollee would owe if the enrollee had received services from a participating provider. The MCO shall apply the out-of-pocket maximum that would have applied had the services been received from a participating provider.

Section 98-5.7 Provider directory requirements.

(a) In addition to the provider directory requirements set forth in Public Health Law 4403 and 4408, when listing a behavioral health provider, the provider directory shall include:

(1) any affiliation with participating facilities certified or authorized by the Office of Mental Health and the Office of Addiction Services and Supports;

(2) information on restrictions on the availability of services from a behavioral health provider. Restrictions on the availability of services means an age limit on the types of patients the behavioral health provider treats or any limits on the types of specific behavioral health conditions that the behavioral health provider treats;

(3) if the behavioral health provider is a facility, the level of care offered by the facility, including inpatient, outpatient, partial hospitalization, and intensive outpatient programs;

(4) the city/town and zip code where the behavioral health provider is located;

(5) whether the behavioral health provider offers services via telehealth; and

(6) if the behavioral health provider is a health care professional, the languages spoken by the health care professional.

(b) With respect to behavioral health providers, the provider directory that is posted on a publicly accessible area of the MCO's website shall be searchable and filterable by behavioral health services provided and conditions treated, level of care offered by a facility, languages spoken, affiliations with participating facilities certified or authorized by the Office of Mental Health or the Office of Addiction Services and Supports, and the city/town or zip code where the provider is located.

(c) In addition to the disclosure requirements set forth in Public Health Law 4408(1)(r), a MCO shall provide the enrollee or the enrollee's designee with a list of behavioral health providers available to treat a specific behavioral health condition within three business days of the request of the enrollee or the enrollee's designee.



(d) A MCO shall verify the accuracy of the information in the provider directory with behavioral health providers at least annually.

(e) A MCO shall review the claims activity of the first six months of the year by September 1 of that year and, for the second six months of the year by March 1 of the following year. If the MCO did not receive any claims from a participating provider of behavioral health services within those periods, the MCO shall confirm whether the provider is accepting new patients and the provider's participation status with the MCO.

(f) A MCO shall have a method available on a publicly accessible area of its website for enrollees, health care providers, and other persons to report errors in the provider directory information. Within 15 calendar days of receipt of reported errors, the MCO shall review the errors reported and ensure that the online provider directory information is accurate.

#### Section 98-5.8 Additional MCO responsibilities regarding network adequacy and access.

(a) A MCO shall have designated staff with sufficient knowledge to help enrollee find participating behavioral health providers that treat the enrollee's specific behavioral health condition. The MCO shall post the contact information for the department or unit, including a telephone number, on a publicly accessible area of its website, that allows the enrollee to access this designated staff directly.

(b) A MCO shall post information on a publicly accessible area of its website describing the appointment wait time standards for behavioral health services and the process to submit an access complaint.

(c)(1) A MCO shall have an access plan that establishes a protocol for monitoring and ensuring access to behavioral health services, outlines how provider capacity is determined, and establishes procedures for quarterly monitoring of capacity and access and for improving access and managing access in times of reduced participating provider capacity. The access plan and associated monitoring protocol shall address the following:

- (i) expected utilization of behavioral health services based on anticipated enrollment and health care needs of the enrollee population;
- (ii) the number and types of health care providers of behavioral health services required to furnish covered behavioral health services, the number and types of providers actively providing behavioral health services within the MCO's network, and the number and types of providers accepting new patients;
- (iii) the collection and monitoring of data on provider-to-enrollee ratios, travel time and distance to participating providers, and appointment wait times;
- (iv) the role of telehealth in providing access to behavioral health services; and
- (v) the ability of the MCO's network of behavioral health providers to meet the cultural and linguistic needs of the MCO's enrollee population.

(2) A MCO shall make the access plan available to the commissioner upon the commissioner's request.

Section 98-5.9 MCO reporting on network adequacy and access.

(a) By December 31, 2026 and annually thereafter, each MCO shall submit to the commissioner a written certification in a form prescribed by the commissioner and signed by an officer of the MCO that confirms the following:

(1) the MCO has an access plan as required by section 98-5.8 of this Part and that such access plan is available upon the commissioner's request;

(2) the MCO has sufficient participating providers in each network used by the MCO to meet the appointment wait time standards as required by section 98-5.5 of this Part, or in instances where there are not sufficient participating providers to meet the appointment wait time standards as required by section 98- 5.5 of this part, that the MCO allows enrollees to obtain behavioral health services from non-participating providers pursuant to section 98-5.6 of this Part;

(3) the number of access complaints received and a description of how the access complaints were resolved, including the behavioral health services requested, the geographic area of the State where the services were requested, the number of approved referrals to non-participating providers made during the prior 12 months pursuant to section 98-5.6 of this Part; and the number of referrals that the MCO did not approve and the reasons why the MCO did not approve the referrals; and

(4) the MCO has performed the provider directory verification required by section 98-5.7 of this Part.

Section 98-6.0 Effective date.

This Part shall take effect July 1, 2025 and shall apply to all policies issued, renewed, modified, or amended on or after such date.

## **REGULATORY IMPACT STATEMENT**

### **Statutory Authority:**

Public Health Law (PHL) section 4403(2) states the Commissioner may adopt and amend rules and regulations to effectuate the purposes and provisions of Article 44, which governs the certification and operational requirements of managed care organizations (MCOs). Public Health Law 4403(5)(b), as amended by Subpart F of part II of chapter 57 of the Laws of 2023 requires the commissioner, in consultation with the Superintendent of Financial Services, the commissioner of the Office of Mental Health, and the commissioner of the Office of Addiction Services and Supports, to propose regulations setting forth standards for network adequacy for mental health and substance use disorder treatment services, including sub-acute care in a residential facility, assertive community treatment services, critical time intervention services, and mobile crisis intervention services, by December 31, 2023.

### **Legislative Objectives:**

To establish network adequacy requirements for mental health and substance use disorder services in order to improve access to behavioral health services.

### **Needs and Benefits:**

The regulation implements the legislative objectives of Chapter 57 by establishing requirements for provider networks used by MCOs that issue comprehensive health insurance contracts in relation to mental health and substance use disorder services. Ensuring meaningful access to mental health and substance use disorder care is vital to

addressing New York’s mental health and substance use disorder crisis. A key component of access is the availability of an adequate number of appropriate providers within a MCO’s network. The Department of Health (“Department”) consulted with the Department of Financial Services (DFS), the Office of Mental Health (OMH), and the Office of Addiction Services and Supports (OASAS) when drafting the regulation. DFS coordinated meetings with numerous stakeholders representing providers, consumers, and health care plans. The regulation sets forth appointment wait time standards for mental health and substance use disorder services. If an enrollee cannot access mental health or substance use disorder services from an in-network provider who can treat the enrollee’s behavioral health condition and is available within the appointment wait time standards, the regulation gives the MCO three (3) business days from receipt of an access complaint to provide the enrollee or the enrollee’s designee with contact information for an in-network provider who can treat the enrollee’s behavioral health condition and is available with the appointment wait time standards. If the enrollee requests an in-person visit rather than a telehealth visit, the in-network provider must also be located within a reasonable distance. If no such in-network provider is available within the appointment wait time standards, the regulation requires the MCO to provide the enrollee with a referral to an out-of-network provider at the in-network cost-sharing, if the out-of-network provider can treat the enrollee’s behavioral health condition, is able to meet the appointment wait time standards, is located within a reasonable distance from the enrollee, and charges are not excessive or unreasonable. The regulation requires the referral to remain in effect until the behavioral health services are no longer medically necessary or the MCO locates an in-network provider that can treat the enrollee’s

behavioral health condition, is able to meet the appointment wait time standards and is located within a reasonable distance, and the enrollee's treatment can be transitioned to the in-network provider and such transition would not be harmful to the enrollee. The regulation requires MCOs to verify information in their provider directories and to include information in the directories on any restrictions concerning the conditions or ages treated by network providers, language spoken by a health care professional, whether the provider offers services via telehealth and, if the provider is a facility, the level of care offered by the facility. The regulation requires MCOs to review claims activity twice each year to identify behavioral health providers who have not submitted claims and to verify their participation status and confirm whether they are accepting new patients. Additionally, the regulation requires MCOs to post certain information on a publicly accessible area of the websites, including a method for enrollees, providers, and other persons to report provider directory errors, a description of the appointment wait time standards, and the process for submitting an access complaint. The regulation requires MCOs to develop an access plan to monitor the utilization of mental health and substance use disorder services and submit an annual certification of compliance to the Commissioner that includes the number of access complaints received by the MCO and a description of how the access complaints were resolved.

## **Costs:**

### **Costs for the Implementation of, and Continuing Compliance with the Regulation to the Regulated Entity:**

A MCO may incur compliance costs to: develop a process to monitor and evaluate access to its network providers; recruit additional mental health and substance use disorder providers for its networks or pay for out of network providers; modify on-line provider directories to ensure that they are searchable and filterable; provide training to staff on the requirements for responding to access complaints; update its website with required information; and submit an annual compliance certification. Some of the compliance costs may impact premium rates charged to enrollees for the commercial line of business. However, certain costs should be minimal because MCOs should already have compliance procedures in place such as the requirement to submit quarterly network reports.

### **Costs to State and Local Governments:**

The new regulation does not impose any compliance costs on state or local governments or health care providers.

### **Costs to the Department of Health:**

The new regulation may impose compliance costs on the Department because the Department will need to monitor MCOs compliance with the new regulation, review annual compliance certifications and update contracts with the Department for Medicaid lines of business. However, any additional costs incurred by the Department should be



minimal because existing personnel are already available to monitor compliance and update contracts necessitated by the new regulation and the Department should be able to absorb the costs in its ordinary budget. In addition, there could be an impact to premiums because if no in-network provider can provide the services within the appointment wait time standards, the regulation requires the MCO to allow the enrollee to access an out-of-network provider at the in-network cost-sharing, if the out-of-network provider can meet the appointment wait time standards.

**Local Government Mandates:**

The new regulation does not impose any program, service, duty, or responsibility upon a county, city, town, village, school district, fire district, or other special district.

**Paperwork:**

MCOs may need to file new policy forms and rates with DFS for commercial lines of business and update contracts with the Department for Medicaid lines of business to comply with the regulation. MCOs will need to develop an access plan that establishes a protocol for monitoring and ensuring access to behavioral health services, outlines how provider capacity is determined, establishes procedures for monitoring of capacity, and establishes procedures for improving and managing access in times of reduced participating provider capacity. MCOs will also need to submit an annual certification of compliance to the Commissioner.

**Duplication:**

The new regulation does not duplicate, overlap, or conflict with any existing state or federal rules or other legal requirements.

**Alternatives:**

The Department consulted with the DFS, OMH, and OASAS when drafting the regulation. The Department and DFS considered requiring MCOs to meet appointment wait time standards of 14 to 28 days, instead of ten business days, for initial mental health and substance use disorder treatment appointments. During discussions with various behavioral health provider associations, providers repeatedly stated that there is a state-wide shortage of providers and an increasing demand for mental health and substance use disorder treatment services. Many providers, including providers who do not participate in MCO provider networks, expressed concern that they would not be able to meet an appointment wait time standard of ten business days, and many providers indicated that appointment wait times can run up to four weeks or longer. However, other states and federally-run exchanges have a ten business-day timeframe for initial appointments, and the ten business-day timeframe is more protective of consumers than a longer timeframe.

The Public Health Law includes a mechanism for an enrollee to go out of network when there is no provider in a MCO's network who can perform the services. That process may require the enrollee to go through multiple levels of appeal. However, the Department chose to require a more streamlined process for MCOs to assist an enrollee

in obtaining an appointment with a provider who meets the appointment wait times which does not necessitate appeals.

The Department considered several different timeframes for MCOs to monitor network capacity and provider access including monthly, quarterly, and annually. The Department added a quarterly timeframe to align with the network adequacy quarterly network submission process.

The Department considered requiring a pre-determined length of time for a referral to an out-of-network provider to be covered, such as sixty (60) or ninety (90) days. However, the interruption of certain behavioral health treatments may cause harm to the enrollee in some circumstances, while in other situations may be more appropriately transitioned to an in-network provider sooner. The Department also considered requiring out-of-network referrals pursuant to the regulation to be effective until the completeness of enrollee's treatment. However, some behavioral health treatments can be very lengthy, lasting years, which would be costly for MCOs and increase premiums. In addition, MCOs currently can transition enrollees to in-network providers in order circumstances where out-of-network referrals are made.

**Federal Standards:**

The regulation does not conflict with any minimum standards of the federal government for the same or similar subject areas.

**Compliance Schedule:**

MCOs will need to comply with the regulation for contracts issued, renewed, modified, or amended on and after January 1, 2025, and will need to submit annual compliance certifications by December 31, 2025.

**Contact Person:**

Katherine Ceroalo  
New York State Department of Health  
Bureau of Program Counsel, Regulatory Affairs Unit  
Corning Tower Building, Rm. 2438  
Empire State Plaza  
Albany, New York 12237  
(518) 473-7488  
(518) 473-2019 (FAX)  
[REGSQNA@health.ny.gov](mailto:REGSQNA@health.ny.gov)

**STATEMENT IN LIEU OF  
REGULATORY FLEXIBILITY ANALYSIS**

No regulatory flexibility analysis is required pursuant to section 202-(b)(3)(a) of the State Administrative Procedure Act. The proposed amendment does not impose an adverse economic impact on small businesses or local governments, and it does not impose reporting, record keeping or other compliance requirements on small businesses or local governments.

## RURAL AREA FLEXIBILITY ANALYSIS

### Types and Estimated Numbers of Rural Areas:

This rule applies uniformly throughout the state, including rural areas. Rural areas are defined as counties with a population less than 200,000 and counties with a population of 200,000 or greater that have towns with population densities of 150 persons or fewer per square mile. The following 44 counties have a population of less than 200,000 based upon the United States Census estimated county populations for 2020 (<https://www.census.gov/quickfacts/>). Approximately 17% of small health care facilities are located in rural areas.

|                    |                    |                     |
|--------------------|--------------------|---------------------|
| Allegany County    | Greene County      | Schoharie County    |
| Broome County      | Hamilton County    | Schuyler County     |
| Cattaraugus County | Herkimer County    | Seneca County       |
| Cayuga County      | Jefferson County   | St. Lawrence County |
| Chautauqua County  | Lewis County       | Steuben County      |
| Chemung County     | Livingston County  | Sullivan County     |
| Chenango County    | Madison County     | Tioga County        |
| Clinton County     | Montgomery County  | Tompkins County     |
| Columbia County    | Ontario County     | Ulster County       |
| Cortland County    | Orleans County     | Warren County       |
| Delaware County    | Oswego County      | Washington County   |
| Essex County       | Otsego County      | Wayne County        |
| Franklin County    | Putnam County      | Wyoming County      |
| Fulton County      | Rensselaer County  | Yates County        |
| Genesee County     | Schenectady County |                     |

The following counties have a population of 200,000 or greater and towns with population densities of 150 persons or fewer per square mile. Data is based upon the United States Census estimated county populations for 2020.

|                 |                 |                 |
|-----------------|-----------------|-----------------|
| Albany County   | Niagara County  | Orange County   |
| Dutchess County | Oneida County   | Saratoga County |
| Erie County     | Onondaga County | Suffolk County  |
| Monroe County   |                 |                 |

## **Reporting, Recordkeeping and Other Compliance Requirements; and Professional Services:**

MCOs, including MCOs in rural areas, may be subject to additional reporting, recordkeeping, or other compliance requirements regarding their network of providers. MCOs will need to develop an access plan that: establishes a protocol for monitoring and ensuring access to behavioral health services, such as monitoring utilization of those services, numbers and types of providers who are actively providing services, collecting data on provider-to-enrollee ratios and appointment wait times, and assessing the cultural and linguistic needs of the enrollee populations. MCOs should also outline how provider capacity is determined, establish procedures for quarterly monitoring of capacity, and establish procedures for improving and managing access in times of reduced participating provider capacity. MCOs will also need to submit an annual certification attesting that they are meeting the requirements outlined in 10 NYCRR 98-5 and to report on the number of access complaints received with a description of how the access complaints were resolved, including the number of approved referrals.

### **Costs:**

The new regulation may impose compliance costs on MCOs, including those in a rural area, to develop a process to monitor and evaluate access to its network providers; recruit additional mental health and substance use disorder providers for its networks; and submit an annual compliance certification. However, any costs should be minimal because MCOs should already have compliance procedures in place.

**Minimizing Adverse Impact:**

This rule uniformly affects MCOs that are located in both rural and non-rural areas of New York State. This rule should not have an adverse impact on rural areas.

**Rural Area Participation:**

The Department of Health participated in virtual meetings with trade associations representing MCOs throughout the state, including those located in rural areas, regarding the proposed regulation. The Department also met with numerous stakeholders representing MCOs, providers, and consumers. MCOs, including MCOs in rural areas, will have an opportunity to participate in the rule-making process by submitting comments after the proposed rule is published in the State Register and on the Department of Health's website.



**STATEMENT IN LIEU OF**

**JOB IMPACT STATEMENT**

A Job Impact Statement for these amendments is not being submitted because it is apparent from the nature and purposes of the amendments that they will not have a substantial adverse impact on jobs and/or employment opportunities.

## ASSESSMENT OF PUBLIC COMMENT

The New York State Department of Health (Department) received comments regarding the amendment to Sections 98-1.13 and addition of Subpart 98-5 of Title 10 of the New York Codes, Rules and Regulations, as re-published in the State Register on December 11, 2024. The Department received comments from associations that represent managed care organizations (MCOs), associations that represent healthcare providers, and advocacy organizations that provide or promote mental health and substance use disorder health care services. Some of the comments the Department received on the revised proposal were previously addressed in the assessment of public comments to the proposed rulemaking and are posted on the Department's website. New comments and the Department's responses are summarized below.

**Comment:** The Department received comments regarding additional appointment types and wait time standards included in the Model Contract that are not included in this regulation.

**Response:** The regulation implements a legislative mandate; however, MCOs with Medicaid lines of business will still be held to contractual obligations for appointment wait time standards for services outlined in the Medicaid Model Contract. No changes were made to the regulation.

**Comment:** The Department received comments regarding the potential challenge of meeting the appointment wait time standards. Additionally, there was a comment on the threshold MCOs will be held to be considered in compliance, stating that it should be at

least 90% taking into consideration the impossibility of meeting the standards all of the time.

**Response:** The Department has existing processes in place for monitoring compliance with access and availability. The Department currently uses a 75% compliance rate and is increasing to 90% to align with the new federal requirement. Surveillance standards are not included in this regulation, so no changes were made.

**Comment:** The Department received a comment asking the reason that an initial appointment can't include a health care professional who is "employed by or contracted with an outpatient facility or clinic"? This language appears to be an unnecessary restriction that will increase the difficulty of meeting already aspirational appointment wait times.

**Response:** Section 98-5.5 of the regulation provides appointment wait time standards for both initial appointments within an outpatient facility or clinic as well as with a health care professional who is not employed by or contracted with an outpatient facility or clinic. No changes were made to the regulations.

**Comment:** The Department received a comment stating that the availability of telehealth appointments within the appointment wait times for initial visits should be sufficient regardless of consumer preference and that telehealth option versus in-person choice should be specific to follow up to hospital or emergency room visit.

**Response:** A MCO may meet the appointment wait times set forth Section 98-5.5 (a) through the use of telehealth unless the enrollee specifically requests an in-person

appointment. The regulations allow for enrollee choice between a telehealth appointment or in-person, so no changes were made to the regulations.

**Comment:** The Department received a comment stating MCOs should be required to develop and maintain internal policies and protocols that outline the plan's compliance with Section 98-5.6 and make them available to the Department upon request.

**Response:** The Department appreciates this comment but did not make changes to these regulations.

**Comment:** The Department received a comment asking for a definition to be included for 'unable to schedule'.

**Response:** The Department appreciates this comment but did not make changes to these regulations.

**Comment:** The Department received a comment to require residential treatment as an additional level of care in the provider directory.

**Response:** The regulation was previously revised to provide examples of level of care offered by a facility to be listed in the provider directory. No changes were made to the regulation.

**Comment:** The Department received comments asking for guidance what a 'reasonable' distance is. In addition, a comment specifically asked about establishing time and distance standards.

**Response:** Network Guidance on time and distance is posted the Departments website and in *Guidelines for MCO Service Delivery Networks*. No changes were made to the regulations.

**Comment:** The Department received a comment requesting clarification in the regulation that the specific names of designated staff are not required to be posted or listed publicly.

**Response:** Due to prior comments requesting clarification of this requirement, the Department made a change in the revised proposed regulation that the MCO shall post the contact information for the department or unit, not the individual name of the designated staff member. No changes were made to the regulation.

**Comment:** The Department received comments regarding determining out of network rates that would be ‘excessive and unreasonable’.

**Response:** It is expected that MCOs will work with out-of-network providers as they do now when there is not an in-network provider to meet the health care needs of the enrollee. It is further expected that a MCO will ultimately reach an agreement with an out-of-network provider. Appointments may be scheduled by telehealth, unless the enrollee specifically requests an in-person appointment, which may facilitate access to a broader spectrum of providers. If the out-of-network referral is not resolved to the enrollee’s satisfaction, the recourse is to submit a complaint to the Department.

**Comment:** The Department received comments seeking clarity regarding the transition back to an in-network provider and grievance process.

**Response:** The regulation adopts a transition of care standard that recognizes the unique nature of behavioral health services. If the MCO and provider deem that a transition would be harmful to the enrollee, the enrollee can continue to see the out of network provider. If the enrollee or enrollee's designee disagrees with determination, they may request an expedited determination or appeal pursuant to Public Health Law section 4408-a or 4904, as applicable.

**Comment:** The Department received a request to consider establishing provider obligations to ensure network adequacy since they are positioned to know their capacity limits and therefore should hold responsibility to aid MCOs.

**Response:** The Department's regulations direct the regulated MCO.

**Comment:** The Department received comments stating that the regulations should be adopted as soon as possible.

**Response:** The Department and New York State Department of Financial Services will align the effective date to July 1, 2025, in case the regulations are adopted on different dates. This change has been made to 98-6.0.

**Comment:** The Department received a comment requesting if a provider is not actively accepting new patients that they are removed from the directory.

**Response:** MCOs are required to include if a provider is currently accepting new patients but should not remove them from the provider directory since they are still part of the MCOs network.

**Comment:** The Department received a comment asking if the MCO is the only entity holding providers responsible for maintaining accurate records relating to provider availability.

**Response:** MCOs are required to ensure provider directories are accurate, including if a provider is accepting new patients. Public Health Law sets forth requirements that MCOs include provider responsibilities regarding the directories in their provider contracts. MCOs are responsible for making sure providers adhere to contractual obligations.

**Comment:** The Department received a comment asking for clarity on what is meant by ‘level of care’ offered by behavioral health provider.

**Response:** The regulation was previously revised to provide examples of level of care offered by a facility to be listed in the provider directory. No changes were made to the regulation.

**Comment:** The Department received a comment regarding MCO verification of the accuracy of the provider directory and that it should be multi-method (e.g., include email and other forms of electronic verification as well as telephonic verification). The regulation should be revised to clarify that these options are available and to eliminate the restriction to written verification.

**Response:** The regulation does not specify the method MCOs should use to verify the accuracy of the information in the provider directory. No changes were made to the regulation.

**Comment:** The Department received a comment that the requirement to review the accuracy of the provider directory ‘at least’ annually is insufficient.

**Response:** The Department added ‘at least’ to the regulation language to specify that the MCO must conduct a review of the provider directory with behavioral health providers at least annually, however the Model Contract and NYCRR 98-1.16 has stricter requirements regarding maintaining and updating their overall Provider Directory that must be continued to be followed. MCOs have discretion to choose the methods to verify their provider directories.

**Comment:** The Department received comments regarding the requirement to review claim activity twice a year as part of the provider directory review and the overlap with current requirements for directory review.

**Response:** The regulation specifies that twice a year the MCO must include a review of claim activity as part of provider directory review for monitoring if a provider is accepting new patients and the provider’s participation status with the MCO. No changes were made to the regulation.

**Comment:** The Department received a comment that the MCO’s website link with designated staff contact information should be submitted to DOH as part of the annual certification for verification of compliance.

**Response:** The Department appreciates this comment and will take it into consideration when developing the written certification document.